



P 3 3 · 6 0 P A G E S , P R I N T A B L E + F I L L A B L E

A Twin Pregnancy Week-by-Week Planner

A sixty-page week-by-week planner for twin pregnancy, from week 6 through delivery. Built around the actual rhythm of a twin pregnancy, more growth scans, more appointments, the chance of earlier delivery, the village you will need, the hospital bag with two of everything. With pages for the feelings most planners don't name, the "what if NICU" page, the postpartum first 24 hours with two, and a partner page.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

A note before you print

Most pregnancy planners are written for one baby. You are not having one baby.

The math of a twin pregnancy is different. You will have more appointments. You will have more growth scans. You will probably gain weight in a curve no chart on the internet quite matches. You will, statistically, deliver earlier than 40 weeks, and "earlier" can mean a lot of things, from "39 instead of 40" to "we are meeting our babies sooner than we thought."

This planner is not a medical book. It is the place to write down what your body did this week, what your provider said at the last visit, what you want to remember to ask at the next, and what you are scared of in the middle of the night.

You can use it cover to cover, or you can skip to the week you are in.

Soothemade

This is a planning and tracking tool, not medical advice. For specific medical concerns, please follow your obstetric provider's guidance. We do not name medications. We do not prescribe a delivery method. The decisions in this pregnancy belong to you and your provider, together.

How to use this planner

- Print on plain paper, or fill on a tablet. The pack is meant to be marked up.
- Start where you are. If you found this at week 22, week 22 is the right place to start. The earlier weeks will still be useful for a second pregnancy, or for someone you know.
- Each weekly page has a “bring to the next appointment” line. Use it. Twin pregnancy moves fast, and the appointment is shorter than the questions you have.
- The companion pages, hospital bag, NICU page, partner page, are at the back. Read them before you need them. Especially the NICU page. Not because anything will go wrong. Because if anything moves, you will be glad you read it once already.
- Keep the planner where you can see it. The bedside table, the kitchen counter, the desk. A planner that lives in a drawer does not get used.

What's different about a twin pregnancy

This is the page to read once and come back to when you forget. It is not a complete medical overview. It is the part the singleton planners leave out.

More appointments. Most twin pregnancies have more frequent prenatal visits than singleton pregnancies, especially from the second trimester on. Your provider will tell you the cadence for you specifically. Plan for the calendar shape, not just the individual visits.

More growth scans. Ultrasounds in twin pregnancy are usually more frequent than in singleton pregnancy. They are checking growth, fluid, and how the babies are doing relative to each other. Most growth scans are reassuring. Some prompt closer monitoring. Your provider will explain what they see.

Different weight curves. The internet's "average weight gain for pregnancy" charts are for one baby. Yours will not look like that. Your provider knows the right range for you.

Earlier delivery is common. A meaningful share of twin pregnancies deliver before 37 weeks. Many deliver between 36 and 38. Some deliver earlier. This is not a failure. This is the most common shape.

NICU is more common than for singletons. Not always. Not even mostly, in some kinds of twin pregnancies. But more common. Reading the NICU page once, ahead of time, is one of the kindest things you can do for the version of you who might walk into that conversation.

Body experience. By the third trimester, two babies take up more room than one. Sleep is harder. Sitting is harder. Walking is harder. Eating without heartburn is harder. The body is doing twice the work.

Emotional experience. Joy and grief sometimes sit beside each other. The grief of "I cannot do what one-baby parents do" is a real grief. So is the grief of "I am scared something will go wrong." So is the joy. They are all allowed to be here at the same time.

The village. Twin parents need help in a way that singleton parents sometimes do not. This is not weakness. This is volume. Start asking now. The village page in the back is the help schedule.

Choosing or confirming your provider

Some twin pregnancies are managed by a general OB. Some by a maternal-fetal-medicine specialist. Some bounce between the two depending on what is happening. There is no wrong path. There is a path your provider is recommending based on what they see.

Questions to ask early:

- ☐ How many twin pregnancies have you managed in the last year?
- ☐ What is the cadence of appointments you expect for me, by trimester?
- ☐ At what week do you typically start more frequent monitoring?
- ☐ What does your group's experience look like with twin deliveries?
- ☐ If something needs closer monitoring, who do I see, and where?
- ☐ What hospital do you deliver at, and what level of NICU does it have?
- ☐ Do you recommend I also see a maternal-fetal-medicine specialist?

If something the provider says does not sit right, you are allowed to ask again. You are allowed to ask in different words. You are allowed to bring a second pair of ears.

What I want to ask my provider this month:

Trimester one

Weeks 6 through 13. The trimester of, you cannot tell anyone yet, and you are unbelievably tired.

Twin pregnancies sometimes show earlier than singleton pregnancies. Your body knows what it is doing before the world does. That is the privacy of it.

This is the trimester of nausea, possibly more than your friends had. The trimester of "I cannot eat anything." The trimester of "I am scared this is not real." The trimester of the first ultrasound, where you find out the news that changes the math.

A common thing twin parents say: "I was so nauseous in the first trimester that I knew something was different before they told me there were two." Common does not mean universal. Yours may be lighter. Yours may be heavier.

The work of this trimester: rest where you can. Eat what you can keep down. Write down the question for the next appointment as soon as it arrives in your head, because you will forget it by Thursday.

Week 6

The week you might have just found out. Or might be about to.

What might be happening: Implantation is complete. Hormone levels are rising fast. Many people feel exhausted and queasy at this point. With twins, hormone levels often rise faster than singleton pregnancies.

A prompt for you:

What did you eat today that you actually kept down. Write it. You will want to repeat it.

I ate: _____

I drank: _____

The thing that helped most today:

The thing that surprised me this week:

Bring to the next appointment:

Week 7

The week of, am I doing this right.

What might be happening: Tiny heartbeats are usually visible on ultrasound starting around now. If you have already had an early scan, you may already know you are carrying two. If you have not, the news may be coming.

A note for the moment you find out it is two:

The minute you find out, you will feel a hundred things. Joy. Fear. The math of childcare. The math of college. The math of your back. The math of your sleep for the next year. All of it. All of it is allowed.

Write down, here, what your first reaction was. You will want to remember.

The minute I found out it was two, I felt:

Bring to the next appointment:

Week 8

The week of the official confirmation, often.

What might be happening: Your provider may confirm the type of twin pregnancy you have. The chorionicity, the amnionicity. This is mostly medical-team language for “how the babies are sharing the space and the placenta.” It matters for monitoring. Your provider will explain what your type means for your appointment schedule.

A page to write down what they told you:

My provider told me my twin pregnancy is:
(in their words)

What that means for my appointment schedule:

What questions I still have:

Bring to the next appointment:

Week 9

The week of, the nausea peaks for many people.

What might be happening: Hormones are at their highest for many. If you are throwing up more than five or six times a day, or you cannot keep liquids down, call your provider. There are options. You do not have to white-knuckle through this.

A prompt:

What is one thing you have stopped doing this week that you used to do every day. Cooking dinner. Going to the gym. Answering texts within an hour. Write it down. It is okay that it stopped.

I have stopped: _____

It is okay that it stopped because:

Bring to the next appointment:

Week 10

The week of, you start to feel slightly more human.

What might be happening: For many, but not all, nausea begins to ease a little. Energy may start coming back in small windows.

A prompt:

Name one small thing that felt good this week. The window open in the morning. The toast that stayed down. The shower at noon.

This week, a small thing that felt good:

The thing that surprised me this week:

Bring to the next appointment:

Week 11

The week before you can sometimes say it out loud.

What might be happening: The end of the first trimester is close. The probability of miscarriage drops significantly after this point, though it never reaches zero. If you have been carrying the news quietly, you may start to think about who you want to tell.

Who to tell, and when:

There is no rule. Some people tell at the first heartbeat. Some wait through the first trimester. Some wait through the anatomy scan. Twin parents sometimes tell earlier because they are showing earlier, and sometimes later because they are scared of telling too soon.

The people I want to tell first:

When I want to tell them:

Bring to the next appointment:

Week 12

The week of, the news goes public for many.

What might be happening: End of the first trimester for many. The risk of miscarriage drops further. Many people share the news around this point.

A prompt for the announcement, if you are making one:

What would the calm version of this announcement say. Not the giddy version. Not the social-media version. The version that is the truth.

The calm version:

Bring to the next appointment:

Week 13

The last week of trimester one.

What might be happening: First trimester closes. You have made it through the most exhausting twelve weeks of your life. Both babies are about the size of a small peach. They are real. You are still tired.

A reflection on trimester one:

The hardest thing about trimester one was:

The thing I am most surprised by:

What I want to bring forward into trimester two:

Trimester two

Weeks 14 through 27. The trimester of, you feel mostly human again, the bump starts to be visible, the appointments increase, and you start to feel them move.

For twin parents, this is often when the pregnancy becomes visible to other people sooner than for singleton pregnancies. Strangers will say things. Some of them helpful. Some of them strange. You do not owe anyone an explanation.

This is the trimester of the anatomy scan, which in a twin pregnancy is usually a longer appointment because they are checking two babies. Bring water. Bring a snack. Bring someone if you can.

The work of this trimester: build the village. Schedule the help. Look at the hospital bag list once, even if you do not pack yet. Read the NICU page even though you are scared to. Tell the partner about the things that are scaring you, before they become bigger.

Week 14

The week you start to feel slightly normal, maybe.

What might be happening: For many, nausea is mostly behind you. Energy starts to come back. The bump may be more visible than a singleton bump would be at this point.

A prompt:

What is something you have been putting off because you were too sick. A phone call. A walk around the block. A meal cooked from scratch. Pick one.

The thing I have been putting off:

This week, I will:

Bring to the next appointment:

Week 15

*T*he week of, are they kicking yet.

What might be happening: Some second-time parents start feeling early movement around this point. For first-time parents, it is usually a little later. With twins, movement is sometimes felt earlier because there are two of them.

A page for noticing:

First time I think I felt something: _____

Where I felt it: _____

What it felt like (in your words): _____

Bring to the next appointment:

Week 16

The week of the maybe-appointment.

What might be happening: Many providers see twin parents around this point for a check-in scan or a regular appointment.

Bring to this appointment (suggested):

- ☐ The list of questions you have written down all month
- ☐ A bottle of water (you may be asked to drink before a scan)
- ☐ A snack (longer scans can mean a hungry afternoon)
- ☐ A partner or friend if you can bring one
- ☐ Your insurance card (twin pregnancies sometimes generate billing questions)

Notes from this appointment:

What they measured:

What they said about it:

What I still want to ask:

Week 17

The week of, the bump is visible to most people now.

What might be happening: The bump is showing. Strangers may comment. Coworkers may notice. The pregnancy has gone public, whether you announced or not.

Scripts for the strangers:

A stranger says, "wow you are huge." You can say:

- "Thanks." (Walk away.)
- "I am carrying twins." (If you want them to know.)
- "Everyone grows differently." (If you want them to back off.)

You do not owe an explanation. You do not have to laugh it off. You do not have to be polite.

The strangest thing a stranger said this week:

What I said back, or wished I had said:

Bring to the next appointment:

Week 18

The week before the anatomy scan, often.

What might be happening: The anatomy scan is usually scheduled around 18 to 22 weeks. For twins, it is a longer appointment. They are checking two babies for the standard list of anatomy markers.

Preparing for the anatomy scan:

My anatomy scan is scheduled for: _____

The questions I want to bring:

1. _____
2. _____
3. _____
4. _____
5. _____

What I want to bring with me:

- ☐ Water bottle
- ☐ Snack
- ☐ Partner / support person
- ☐ Comfortable clothes (the scan can take an hour or more)
- ☐ This planner

If they find something that concerns them, what is my plan for the next 24 hours?

Bring to the next appointment:

Week 19

The week of the scan, often. Or just after.

What might be happening: If you had the anatomy scan, you may have new information. Most anatomy scans in twin pregnancy are reassuring. Some prompt closer monitoring or a referral. Whatever happened, it is okay to need a day or two to process.

A page for the scan:

The anatomy scan was on: _____

What they told me about Baby A:

What they told me about Baby B:

What I am sitting with after this scan:

What I still want clarified:

Bring to the next appointment:

Week 20

The halfway week.

What might be happening: Halfway through, or close to it, depending on when you deliver. Many twin parents do not reach 40 weeks. So halfway for you may be earlier than the calendar suggests.

A reflection:

The thing I have learned about myself in the first half:

The thing I am most ready to have help with:

What I want the second half to look like:

Bring to the next appointment:

Week 21

The week of, you can feel two distinct movements sometimes.

What might be happening: Movement is often clearer now. Some parents start to distinguish Baby A from Baby B by where the kicks land.

A page for movement:

Baby A tends to be: high / low / right / left
(circle one or more)

Baby B tends to be: high / low / right / left

The thing they do at night that wakes me:

The thing they do in the morning:

Bring to the next appointment:

Week 22

The week of, I am tired in a way that surprises me.

What might be happening: Twin pregnancy fatigue is a real thing. The body is doing twice the work. Sleep is becoming harder.

A permission slip:

You are allowed to nap in the middle of the day. You are allowed to say no to events you said yes to in trimester one. You are allowed to ask your partner to take over a task you usually do. You are allowed to be tired without explaining it.

Bring to the next appointment:

Week 23

*T*he week of, the kicks have a rhythm.

What might be happening: Many parents start to notice a pattern in when the babies are most active. Often after meals. Often when you lie down. Often at 11pm right when you wanted to sleep.

A page for the pattern:

Most active time of day: _____

Quietest time of day: _____

What seems to wake them up:

What seems to calm them down:

Bring to the next appointment:

Week 24

The week of, the appointments are starting to pick up.

What might be happening: For many twin pregnancies, the cadence of appointments increases around the second half of the second trimester. Growth scans become more frequent.

A page for the new rhythm:

My new appointment cadence (from my provider):

How I will get to the appointments:

Who will come with me, if anyone:

How I will keep my job + appointments balanced:

If I am self-employed, the schedule I am building:

Bring to the next appointment:

Week 25

The week of, the bump is starting to make sitting harder.

What might be happening: Many people start feeling more discomfort sitting upright for long stretches. Some can no longer sleep on their back.

A prompt:

What is one piece of your daily routine that has stopped working. The morning commute. The standing desk. The hour-long meeting. Write it down and figure out the alternative.

What stopped working:

The alternative:

Bring to the next appointment:

Week 26

The week of, are you starting to think about delivery.

What might be happening: Many providers start asking about delivery preferences and birth-plan considerations in the second half of pregnancy. For twin parents, this conversation often happens earlier than for singletons.

A page to start thinking:

The hospital I am planning to deliver at:

The hospital tour, if I have done one:

What I know about my hospital's twin-delivery experience:

What questions I want to ask about the delivery:

What feelings I have about the delivery right now:

Bring to the next appointment:

Week 27

The last week of trimester two.

What might be happening: End of the second trimester. The babies are real. The bump is real. The exhaustion is real. The third trimester is the one where many things speed up.

A reflection on trimester two:

The hardest thing about trimester two was:

The thing I am most grateful for:

What I want trimester three to look like:

What growth scans look at, in plain language

Growth scans in a twin pregnancy are usually more frequent than in a singleton pregnancy. They are checking some specific things. This page is so you can understand what your provider is saying, not so you can interpret the scan yourself.

Estimated fetal weight. They use measurements to estimate each baby's weight. These are estimates, not exact. Babies are growing at a rate, not a snapshot.

Growth percentile. They compare each baby's measurements to a curve. Babies are not "supposed to" be at a specific percentile. They are tracked over time. A baby growing along the same percentile is doing what they are doing.

Discordance. When one baby is meaningfully larger than the other. This is checked but does not by itself mean anything is wrong. Your provider will tell you what their threshold is and what they would do at that threshold.

Amniotic fluid. Both babies, separately. Each baby has their own fluid in some twin pregnancies, depending on type.

Doppler studies. Blood flow in the cord. Sometimes done routinely. Sometimes only if there is a concern.

Cervical length. Sometimes checked at growth-scan appointments. A short cervix earlier than expected can prompt closer monitoring.

What "we want to see you sooner" means. It usually means closer monitoring, not necessarily that anything is wrong. Ask your provider, in plain language, what they are watching and why.

What is NOT for you to interpret:

- Specific measurements on the report

- Whether something is “okay” or “not okay”
- What the next step should be

That is the provider’s job. Your job is to ask. Their job is to explain.

At the last growth scan, my provider said:

What they are watching:

What they want me to do differently:

Trimester three

Weeks 28 through delivery. For many twin parents, delivery comes between 36 and 38 weeks, though it can be earlier or later.

This is the trimester of appointments every week, then every few days. The trimester of the hospital bag actually getting packed. The trimester of the nursery getting set up, or not getting set up because you are too tired. The trimester of the realization that you are about to meet them.

It is also the trimester of the most physical discomfort. Sleep is hard. Walking is hard. Heartburn is loud. You may not be able to eat a full meal.

The work of this trimester: prepare for delivery without obsessing over the plan. Read the NICU page (page near the back) once. Pack the hospital bag at week 32 or 33, not week 38. Have the conversation with your partner about the first 24 hours. Set up the village. Read the postpartum page.

Week 28

The first week of trimester three.

What might be happening: Glucose screening usually happens around this point. Iron levels are often checked. Your provider may start monthly or more frequent visits.

The third-trimester appointment cadence:

My provider's schedule for me (starting now):

Who is going with me, and to which ones:

How long each appointment usually takes:

Bring to the next appointment:

Week 29

The week of, can you walk a city block.

What might be happening: Many parents notice walking becomes harder around this point. The bump is large enough to shift balance and slow pace.

A permission slip:

*You are allowed to take elevators. You are allowed to sit down in stores.
You are allowed to ask for a chair at events. You are allowed to leave
events early.*

Bring to the next appointment:

Week 30

*T*he week of, three quarters.

What might be happening: Roughly three-quarters of the way through, depending on when you deliver. Most twin parents will deliver between 36 and 38 weeks, so for many, this is closer to two-thirds.

A page for the math:

If I deliver at 36 weeks, I have ____ weeks left.

If I deliver at 38 weeks, I have ____ weeks left.

What I want to do with this stretch:

What can wait until after:

Bring to the next appointment:

Week 31

The week of, the practical preparations.

What might be happening: Many parents start setting up the practical pieces this month. Two of some things, one of others.

The practical-set-up checklist:

- ☐ Two car seats installed
- ☐ Two newborn-size everything (sleep sacks, diapers, swaddles)
- ☐ Two sets of bedding for the bassinets
- ☐ One changing setup (you do not need two. one with everything is enough)
- ☐ One pediatrician chosen
- ☐ One pediatrician appointment scheduled for 1-2 days after discharge
- ☐ Hospital bag started (see page below)
- ☐ The partner's bag started
- ☐ A go-bag for the partner with multiple changes of clothes
- ☐ A list of who to call when labor starts
- ☐ A plan for the older sibling, if there is one
- ☐ A plan for the pets
- ☐ A list of meals in the freezer (or a meal-train link sent out)
- ☐ A plan for laundry, the first week
- ☐ A backup plan if you deliver before the original plan was ready

The thing I am most worried about not having done:

Bring to the next appointment:

Week 32

The week to pack the hospital bag, recommended.

What might be happening: Many providers recommend having the hospital bag packed by 32 to 34 weeks. Twin parents in particular benefit from packing early.

(See the hospital bag for twins page near the back of this planner.)

A note to self:

Hospital bag packed: ☐ Yes ☐ No
Partner's bag packed: ☐ Yes ☐ No
Car seats installed and inspected: ☐ Yes ☐ No
Pediatrician picked: ☐ Yes ☐ No
Going-home outfits (two): ☐ Yes ☐ No
A list of phone numbers on paper: ☐ Yes ☐ No

Bring to the next appointment:

Week 33

The week of, you may not be able to eat a full meal.

What might be happening: Heartburn, fullness, and reduced stomach capacity are common in late twin pregnancy. Small meals every few hours often work better than three larger meals.

A meal plan that often works:

- Small breakfast in bed before sitting up.
- Mid-morning snack.
- Small early lunch.
- Mid-afternoon snack.
- Small dinner.
- Bedtime snack so you do not wake up at 3am hungry.

What is working for my eating right now:

What is not working:

Bring to the next appointment:

Week 34

The week of, the babies could come early.

What might be happening: From around now, if delivery happened, the babies would be considered "late preterm" rather than very early. This does not mean delivery will happen. It just shifts what early delivery might look like, if it happened. Talk to your provider about your specific situation.

A page for the fear:

What I am scared of right now:

What my provider has actually said to me about this:

The difference between the two:

Bring to the next appointment:

Week 35

The week of, the appointments are very frequent.

What might be happening: Many twin pregnancies have weekly appointments by now, sometimes more often.

A note about appointment fatigue:

You may be tired of going to the doctor. That is real. Bring snacks. Bring a book or a podcast. Bring a partner if you can. Tell the front desk you need to sit down. Ask for help walking from the car if you need it.

The thing I have learned about myself from all these appointments:

Bring to the next appointment:

Week 36

The week many twin parents deliver.

What might be happening: Many twin parents deliver around this point. Some sooner. Some later. Your provider will have talked with you about what is expected for your specific pregnancy.

A page for the maybe:

What my provider has said about my timeline:

What I am hoping for:

What I am preparing for:

The first call I will make when labor starts:

The second call:

The third call:

Bring to the next appointment:

Week 37

The week called “early term” for twins, often.

What might be happening: A meaningful share of twin pregnancies have delivered by now. Some providers recommend delivery around this point for certain types of twin pregnancies. Your provider will have discussed your specifics.

A page for the conversation with your provider:

What my provider has recommended about timing:

What I want to ask before any planned delivery:

Bring to the next appointment:

Week 38

If you are still pregnant.

What might be happening: Some twin pregnancies do continue past 37 weeks. Your provider is the one calling the shots on what is recommended.

A page for the unexpected stretch:

The thing I did not expect about still being pregnant at 38 weeks:

What I am doing with the extra time:

Bring to the next appointment:

Week 39

Still pregnant.

What might be happening: Rare but possible to be carrying twins this far. Your provider has a plan.

Notes:

Week 40

Full term.

What might be happening: Most twin parents have delivered before this point. If you are reading this and you are 40 weeks, you have an unusual pregnancy and your provider has talked to you about it.

What I want to remember about reaching 40 weeks with twins:

When delivery moves earlier

This page exists for the version of you whose plan changes. Not because something is going wrong. Because twin deliveries often move on a different timeline than the calendar suggests.

If your provider says the delivery date has moved:

- Ask them to explain, in plain language, why.
- Ask what they are watching for and what they would want to see.
- Ask what the immediate next steps are.
- Ask who you should call if you have follow-up questions in the next 24 hours.
- Ask if you can bring your partner to the next appointment.

The date my provider has moved the plan to:

The reason they gave:

What I want to make sure happens between now and then:

What I want to tell my partner right now:

What I want to tell myself right now:

A reminder: A change in timing is not a failure. It is information. Your body is being read by people whose job is to read it. The plan can change. You can adjust.

Hospital bag for twins

Twin hospital bags are not twice the size of singleton bags. They are slightly larger, with two of a few things. Pack this around week 32.

For the parent:

- ☐ Photo ID + insurance card
- ☐ A copy of the birth-plan-summary page (see P14 if you have it)
- ☐ Phone + charger (long cable, the outlet may be far)
- ☐ Toiletries in a small bag
- ☐ Comfortable going-home clothes (loose, dark, soft)
- ☐ A robe (the hospital one is fine, but a soft personal one is better)
- ☐ Slippers with backs (not slip-ons that fall off)
- ☐ Snacks that survive a backpack (granola bars, dried fruit, crackers)
- ☐ A water bottle with a straw (you will want to drink lying down)
- ☐ A nursing or feeding tank if you are planning to breastfeed
- ☐ Lanolin or breastfeeding-friendly nipple cream if recommended
- ☐ Earplugs and eye mask (the hospital is loud and bright)
- ☐ A pen (for paperwork)
- ☐ This planner (for postpartum first 24 hours notes)

For each baby:

- ☐ Two newborn outfits (one is a backup)
- ☐ Two going-home outfits
- ☐ Two newborn hats
- ☐ Two pairs of socks or scratch mittens
- ☐ Two swaddles or sleep sacks
- ☐ Two muslin blankets
- ☐ A diaper rash cream the pediatrician suggests
- ☐ Their own going-home photo outfit if you want one

For the partner:

- ☐ Photo ID
- ☐ Their own toiletries
- ☐ Multiple changes of clothes (they may be there for days)
- ☐ Snacks they like (the hospital food does not work for everyone)
- ☐ A book or something to do during quiet stretches
- ☐ A pillow from home (the hospital recliner has no good pillow)
- ☐ Phone + charger
- ☐ A list of people to call after delivery

For the second car seat:

- ☐ Installed in advance (do not try to install at the hospital)
- ☐ Inspected (most fire stations or police stations do this free)
- ☐ Practice once with a doll or stuffed animal
- ☐ Know how to recline the back seat or front seat to install easily

The thing I keep forgetting to add to the bag:

The “if NICU” page

This page is not a prediction. It is a preparedness. Reading it once, before you need it, is one of the kindest things you can do for the version of you who might walk into the NICU conversation.

Twins have higher NICU rates than singletons. Not always. Not even mostly, depending on the type. But meaningfully more often.

Some reasons for a NICU stay after twin birth:

- Earlier delivery, even by a couple of weeks, can mean the babies need help with breathing or feeding while they finish growing.
- Sometimes one baby goes to NICU and the other does not. This is a hard thing to be ready for.
- Sometimes both go.
- Sometimes neither does.

Things to know in advance:

- The hospital you are delivering at has a NICU at a specific level. Ask your provider what level it is. If it is not the level you need, the babies may be transferred to a different hospital.
- You may be discharged before your babies. This is more common with twins.
- You will be able to visit. Most NICUs welcome parents 24 hours a day.
- You can ask for skin-to-skin even with monitors attached, in many cases. Ask.
- You will be encouraged to pump or to bring milk if you are breastfeeding. NICU lactation support is usually available.
- A NICU stay does not predict the rest of your child’s life. Babies who spend weeks in NICU grow into kids who do not remember it.

If it happens:

The level of NICU at my hospital: _____

Who I will call first:

Who will manage the visitor schedule:

Who will handle work communication:

What I want my partner to remember to bring me at the hospital:

What I want to remember about myself:

If a NICU stay happens, P28 NICU Parent Planner is the next book to reach for. We will not pretend that the planner makes the experience easy. But it gives you somewhere to write things down.

Postpartum: first 24 hours with two

The first 24 hours after delivery, especially with twins, is a blur. This page is to be filled in slowly, after, when you have a moment.

Right after delivery:

Time Baby A was born: _____

Time Baby B was born: _____

Weight of Baby A: _____

Weight of Baby B: _____

Who held each baby first:

Baby A: _____

Baby B: _____

The first thing I noticed about Baby A:

The first thing I noticed about Baby B:

What I want to remember about the minute I saw them:

The first night:

Who was sleeping where:

The hardest thing about the first night:

The thing that surprised me:

Partner page

This page is for the support person. Read it together if you can.

The shift in workload.

You will not be the parent who carried them. You will be the parent who shows up for everything else. That is a real, full job.

The early weeks with twins, the support person is often doing:

- Diapers and burping for the baby the parent is not currently feeding
- Bottle warming or formula prep
- Pumping setup and cleaning, if breastfeeding
- Laundry, dishes, groceries, meals
- The phone calls to family
- The schedule of who is visiting and when
- The morale of the parent who delivered

The signal to watch for.

Two weeks in, three weeks in, six weeks in, watch your partner. Postpartum depression and anxiety are more common in twin parents than in singleton parents. The signal is not always sadness. It can look like withdrawal. It can look like anger. It can look like flatness.

If any of these is true, gently route to the provider:

- ☐ They have not eaten a full meal in 48 hours
- ☐ They are crying for more than 20 minutes at a stretch, more than once a day
- ☐ They cannot sleep when the babies are sleeping
- ☐ They have said something like "I am not built for this" or "they would be better off without me"

- ☐ They are not interested in the babies in a way that feels new
- ☐ Your gut is telling you something is wrong

You are allowed to call the provider on your partner's behalf. You do not need their permission.

What you can say to them.

- "You are doing the hardest job I have ever seen."
- "I will take this one. Go lie down."
- "What do you need that you have not asked for."
- "I am calling the provider. We will figure this out."

Village help schedule

Twin parents need help in a way singleton parents sometimes do not. This is not weakness. This is volume.

The help-schedule template:

Week 1 (first week home):

Monday:

Morning: _____

Afternoon: _____

Evening: _____

Tuesday:

Morning: _____

Afternoon: _____

Evening: _____

Wednesday:

Morning: _____

Afternoon: _____

Evening: _____

Thursday:

Morning: _____

Afternoon: _____

Evening: _____

Friday:

Morning: _____

Afternoon: _____

Evening: _____

Saturday:

Morning: _____

Afternoon: _____

Evening: _____

Sunday:

Morning: _____

Afternoon: _____

Evening: _____

What “help” looks like:

It does not look like holding babies for photos. It looks like:

- Bringing groceries
- Doing dishes
- Folding laundry
- Walking the dog
- Sitting with the older sibling
- Bringing the parent a meal
- Driving to the pediatrician appointment
- Picking up a prescription

The five people I am asking to come, and the day I am asking for:

1. _____ for _____
2. _____ for _____
3. _____ for _____
4. _____ for _____
5. _____ for _____

The “I am scared” page

You are allowed to be scared. The internet will tell you to be excited. Excitement and fear are sometimes the same feeling with different names.

What I am scared of, right now:

Who I have told:

What they said back:

The version of me on the other side of all this would say to the version of me sitting here right now:

A final letter

You are about to have two of them.

Most days, this will be ordinary. The diapers. The pump. The bottle. The bath. The night feed. The morning feed. The slow rebuilding of your body. The grocery run with two car seats. The school of figuring out which baby cries which cry.

Some days will not be ordinary. They will be bigger. The first night home. The first appointment after delivery. The first time both of them cry at the same time and you are alone. The first time you remember you used to be a person without children.

You are allowed to be tired. You are allowed to be scared. You are allowed to grieve the easier version of pregnancy you imagined before you knew it was two.

You are also allowed to be in love. With both of them. With the shape they make when they are sleeping near each other. With the quiet at 4am when you are the only one awake.

The planner runs out at delivery. The other Soothemade products take over from there.

You are going to be a good parent. You already are.

Soothemade



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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